



Name: Mads Raye Robertson-Hagen | DOB: 11/4/2004 | MRN: 7-262-150 | PCP: Jenna Grubisch, APRN, C.N.P., D.N.P.



200 First Street SW
Rochester, Minnesota 55905

507-284-2511
mayoclinic.org

Madison R. Robertson-Hagen "Mads"

January 22, 2026
RE: Madison Raye
Robertson-Hagen
MC#: 7-262-150
DOB: 11/4/2004

Dear Madison R. Robertson-Hagen "Mads":

This letter is to document the Transgender and Intersex Specialty Care Clinic's (TISCC) clinical recommendation for you to pursue medically necessary and clinically appropriate gender affirming surgery, specifically, gender affirming chest surgery (mastectomy) for the management of gender incongruence. You initially met with me on 8/1/2024 for a comprehensive diagnostic assessment and have continued to meet with TISCC Behavioral Health semi-annually since our initial assessment. Additionally you have met with your healthcare provider(s) at TISCC for ongoing care related to gender incongruence quarterly since 2024 .

I, Leonardo Candelario Perez PhD, am a Licensed Clinical Psychologist licensed by the Minnesota Board of psychology. I have completed a post doctorate in human sexuality and gender health at University of Minnesota's Institute for Sex and Gender Health. Currently, I practice as a psychologist at Mayo Clinic's Transgender and Intersex Specialty Care Clinic. I have over 10 years of clinical experience working with patients diagnosed with gender dysphoria.

Additionally, I can diagnose co-existing mental health concerns and distinguish these from gender dysphoria. Finally, I can assess a person's capacity to consent for treatment. As part of a multidisciplinary team, I frequently participate in engagement with other health care professionals from different disciplines within

the field of transgender health for consultation. I have specifically coordinated with the TISCC's team that include MDs, DNPs, PAs, Psychiatry, Psychology and Social Work on a bi-weekly basis.

You meet the DSM-5's (Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition) diagnostic criteria for gender dysphoria/incongruence, which is associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning. Any other *mental health conditions have been assessed and deemed to be* well controlled currently, as evidenced by participation in individual therapy as well as adherence to other health related recommendations made by other/past health care providers. Additionally, you have engaged in all therapeutic recommendations for management of gender incongruence. Despite these interventions you have continued to experience significant distress related to feminine features leading to persistent misgendering, anxiety and social avoidance. The following functional impairments are contributing to this distress: , , Physical discomfort and limitations resulting from chest binding, such as pain, restricted breathing, skin irritation, reduced exercise capacity, and difficulty performing work or household tasks safely or comfortably. Persistent psychological distress, including anxiety, dysphoria, and difficulty engaging in social, vocational, or public activities due to chest-related gender incongruence. Social Functioning: Avoiding public settings and social interactions due to frequent misgendering and fear of discrimination or harassment.

You report heightened anxiety when engaging in work, educational, or community environments..

You provided informed consent throughout your time receiving care for pursuing gender affirming chest surgery (mastectomy). You demonstrate capacity to make fully informed decisions which include understanding the risks and benefits of surgery as well as its impact on fertility and sexual function.

Our records indicate that you meet the readiness criteria for genital surgery recommended by the Standards of Care for the Health of Transsexual, Transgender, and Gender Nonconforming People published by the World Professional Association for Transgender Health (WPATH).

Our recommendation to support your surgery for the management of gender incongruence is based on you having met the following criteria:

- Gender incongruence is marked and sustained;
- Meets diagnostic criteria for gender incongruence prior to gender-affirming surgical intervention in regions where a diagnosis is necessary to access health care;
- Demonstrates capacity to consent for the specific gender-affirming surgical intervention;
- Understands the effect of gender-affirming surgical intervention on reproduction are irreversible and they have explored reproductive options; additionally patient understand that reversal of breast/chest and genital

surgical treatment are not eligible for coverage prior to providing informed consent for this surgery

- Other possible causes of apparent gender incongruence have been identified and excluded; Mental health and physical conditions that could negatively impact the outcome of gender-affirming surgical intervention have been assessed, with risks and benefits have been discussed;
- Stable on their gender affirming hormonal treatment regime (which may include at least 6 months of hormone treatment or a longer period if required to achieve the desired surgical result, unless hormone therapy is either not desired or is medically contraindicated).
- Housing is currently stable and you have a well developed support network and plan for post-operative recovery; allowing for compliance with all post operative expectations including short and long term follow ups.

Our records indicate that you were assigned female at birth and you identify as non-binary Transmasculine. You began gender affirming hormone therapy in 2024 at TISCC and you have socially presented yourself in the gender that is most congruent with your gender identity when safe to do so for over 12 months. You have expressed your gender through the use of pronouns and outwardly expressing your gender identity through clothing and hair choices along with using your affirmed name in daily interactions. Our records indicate that you do not use nicotine products and that you plan to continue to abstain from nicotine.

The Transgender and Intersex Specialty Care Clinic's recommendations for you include gender affirming chest surgery (mastectomy) as part of a medically necessary treatment plan for the management of gender incongruence. The medically necessary treatment is an acceptable standard of clinical practice that is based on credible scientific clinical evidence and professional consensus and together indicates that the treatment is not experimental and is medically necessary and essential for the management of gender incongruence and to sustain functioning, health, and wellbeing. Denial or omission of this recommended treatment will likely result in the deterioration of functioning, health, and wellbeing. This condition is associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning. Support for the medically necessary treatment includes professional consensus from the American Medical Association and the World Professional Association for Transgender Health.

In addition, the United States Department of Health and Human Services has issued the Nondiscrimination in Health Programs and Activities rule which prohibits the denial of health care or coverage based on an individual's sex, including gender identity and sex stereotyping.

This letter of support is valid for one year, from the date of support, and expires on 1/20/2027. If you have any questions, or are in need of additional information for coordination of care, please do not hesitate to contact me at 507-266-3769.

Sincerely

A handwritten signature in black ink that reads "L. Candelario Perez". The signature is written in a cursive, flowing style.

Leo Candelario Perez, Ph.D., L.P.
Division of Endocrinology in Rochester, Minnesota
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